ROYAL FITNESS CLUB

RFC PREMIUM GUIDE #19

PCT POST CYCLE THERAPY COMPLETE BIBLE

Complete Guide to HPTA Recovery After Every Cycle

6 Chapters · Essential Reading · All Cycle Users · Medical Reference

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DISCLAIMER

For educational and harm-reduction purposes only. Not medical advice. Consult a qualified healthcare professional before starting any protocol. Individual results vary.

CHAPTER 1 — HPTA SUPPRESSION PHYSIOLOGY

"PCT does not restore your testosterone — it restores your body's ability to make testosterone. The distinction matters because it changes how you approach the protocol."

Anabolic steroid use suppresses the Hypothalamic-Pituitary-Testicular Axis (HPTA) through negative feedback. Exogenous androgens signal to the hypothalamus and pituitary that testosterone production is adequate, causing a reduction and eventual cessation of GnRH, LH, and FSH secretion. Without LH, the Leydig cells in the testes reduce and eventually cease testosterone production. PCT uses SERMs to block estrogen receptors in the hypothalamus/pituitary, tricking them into perceiving low estrogen and increasing GnRH → LH → FSH signalling.

Compound Duration	Suppression Severity	Recovery Without PCT	Recovery With PCT
< 4 weeks, mild	Mild	2–4 weeks	1–2 weeks
6–10 weeks, moderate	Moderate	3–6 months	4–6 weeks
12–16 weeks, standard	Significant	4–8 months	4–6 weeks
20+ weeks, suppressive	Severe	6–12 months+	6–10 weeks
Blast and cruise (no PCT)	Complete	Possible permanent deficiency	TRT evaluation recommended

CHAPTER 2 — SERM PROTOCOLS

Protocol	Clomid Week 1	Clomid Week 2	Clomid Week 3	Clomid Week 4	Notes
Mild PCT	None	None	None	None	Mini PCT: Nolvadex 20mg × 4 wks only
Standard PCT	50mg/day	50mg/day	25mg/day	25mg/day	Nolvadex 40/40/20/20 alongside
Aggressive PCT	100mg/day	50mg/day	50mg/day	25mg/day	+ Nolvadex 40/40/20/20
Extended PCT	50mg/day	50mg/day	25mg/day	25mg/day	Continue Nolvadex weeks 5–6: 20mg

Clomid vs Nolvadex: Why Use Both?

SERM	Receptor Selectivity	Primary PCT Use	Main Side Effect
Clomid (Clomiphene)	Hypothalamus/pituitary ERα antagonist	LH/FSH stimulation	Vision disturbances (1–2%), mood
Nolvadex (Tamoxifen)	Pituitary ERα antagonist + breast tissue	LH stimulation + breast tissue protection	Mood changes, joint stiffness (rare)
Combined	Dual mechanism	Maximum axis stimulation	Monitor vision carefully on Clomid

CHAPTER 3 — HCG: BRIDGE & BLAST USES

"HCG keeps the testes "awake" during a cycle so PCT has less work to do. Think of it as maintaining the engine while the ignition is temporarily bypassed."

Human Chorionic Gonadotropin (HCG) mimics LH at the Leydig cell receptor. Used during a cycle, it prevents testicular atrophy and maintains intratesticular testosterone production, making PCT faster and more complete. It does not, however, restore the HPTA axis itself — SERMs are still required post-HCG.

HCG Protocol	Dose	Frequency	Duration	Purpose
On-cycle maintenance	250 IU	Twice weekly	Throughout cycle	Prevent atrophy, maintain fertility
Pre-PCT blast	500 IU	Every other day	2–3 weeks pre-PCT	Resensitise Leydig cells
Post-cycle bridge	1000 IU	Every other day	10–14 days	Restart production before SERMs
TRT co-administration	250–500 IU	Twice weekly	Ongoing with TRT	Preserve fertility and testicular volume

■ ***Stop HCG at least 3 days before starting SERMs (Clomid/Nolvadex). Running both simultaneously causes HCG to compete with your natural LH signal.***

CHAPTER 4 — PCT FOR SPECIFIC COMPOUNDS

Compound(s) Used	Wait Before PCT	PCT Protocol	Duration
Testosterone Enanthate only	14–18 days	Nolvadex 40/40/20/20	4 weeks
Test E + Deca (NPP)	10–14 days (NPP)	Clomid 50/50/25/25 + Nolvadex	6 weeks
Test E + Deca long-ester	21+ days	Aggressive PCT 6–8 weeks	6–8 weeks
Test + Tren + Mast	7–10 days (prop esters)	Clomid + Nolvadex + Caber (2 weeks)	4–6 weeks
Oral only (Var, Winstrol)	7 days	Mini PCT: Nolvadex 20mg × 4 wks	4 weeks
SARMs	3–5 days	Nolvadex 40/40/20/20 (all SARMs)	4 weeks

CHAPTER 5 — RECOVERY OPTIMISATION

Support Supplement	Dose	Purpose	Duration
Vitamin D3	5000 IU/day	Leydig cell receptor support	Throughout PCT + 3 months after
Zinc	25–30mg/day	Testosterone synthesis co-factor	Throughout PCT + 3 months
Ashwagandha KSM-66	600mg/day	Cortisol reduction, T support	Throughout PCT + 3 months
Omega-3	4g/day	Anti-inflammatory, lipid recovery	Throughout PCT + ongoing
Magnesium glycinate	400mg before bed	Sleep quality, HPTA support	Throughout PCT
Tribulus (for libido)	1500mg/day	Mild LH stimulation, libido support	Weeks 3–4 of PCT only

CHAPTER 6 — WHEN PCT FAILS

"A failed PCT is not a catastrophe — it is medical information. It tells you the HPTA needs professional support. Act on it."

PCT failure is defined as testosterone remaining below 300 ng/dL at 8 weeks post-PCT completion, or below 80% of pre-cycle baseline. This requires medical evaluation.

- Step 1: Repeat blood work at 8 and 12 weeks post-PCT — confirm it is persistent, not temporary
- Step 2: Consult an endocrinologist — bring your full blood work history
- Step 3: HCG stimulation test to determine if issue is testicular or hypothalamic/pituitary
- Step 4: Extended SERMs trial under medical supervision (6–12 weeks)
- Step 5: If testosterone remains low and symptoms present — TRT is a legitimate therapeutic option
- Do NOT immediately restart another cycle — suppression on already-suppressed HPTA is dangerous

■ ***Many men who "need TRT" after cycles actually have recoverable HPTA dysfunction that was not given enough time. 6–12 months is the full recovery window — not 6 weeks.***